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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
## WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

**Notes:**

**Patient:** Mrs Priya Sharma
**DOB:** 08.05.53 (Age 60)
**Residence:** 71 Seaside Street, Newtown

**Social Background:** Married 40 years – 3 adult children, 5 grandchildren (overseas). Retired (clerical worker).

**Family History:** Many relatives with type 2 diabetes (NIDDM)
Nil else significant

**Medical History:** 1994 – NIDDM
Nil significant, no operations
Allergic to penicillin
Menopause 12 yrs
Never smoked, nil alcohol
No formal exercise

**Current Drugs:** Metformin 500mg 2 nocte
Glipizide 5mg 2 mane
No other prescribed, OTC, or recreational

**29/12/13**

**Discussion:** Concerned that her glucose levels are not well enough controlled – checks levels often (worried?)
Attends health centre – feels not taking her concerns seriously
Recent blood sugar levels (BSL) 6-18
Checks BP at home
Last eye check October 2012 – OK
Wt steady, BMI 24
App good, good diet
Bowels normal, micturition normal

**O/E:** Full physical exam: NAD
BP 155/100
No peripheral neuropathy; pelvic exam not performed
Pathology requested: FBE, U&Es, creatinine, LFTs, full lipid profile, HbA1c
Medication added: candesartan (Atacand) tab 4mg 1 mane
Review 2 weeks



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| | |
|---|---|
| **05/01/14** | Pathology report received:<br>FBE, U&Es, creatinine, LFTs in normal range<br>GFR > 60ml/min<br>HbA1c 10% (very poor control)<br>Lipids: Chol 6.2 (high), Trig 2.4, LDLC 3.7 |
| **12/01/14** | Review of pathology results with Pt<br>Changes in medication recommended<br>Metformin regime changed from 2 nocte to 1 b.d.<br>Atorvastatin (Lipitor) 20mg 1 mane added<br>Glipizide 5mg 2 mane<br>Review 2 weeks |
| **30/01/14** | Home BP in range<br>Sugars improved<br>Pathology requested: fasting lipids, full profile |
| **06/02/14** | Pathology report received: Chol 3.2, Trig 1.7, LDLC 1.1 |
| **10/02/14** | Pathology report reviewed with Mrs Sharma<br>Fasting sugar usually in 16+ (high) range<br>Other blood sugars 7-8<br>Refer to specialist at Diabetes Unit for further management of sugar levels |

## Writing Task:

Using the information in the case notes, write a letter of referral to Dr Smith, an endocrinologist at City Hospital, for further management of Mrs Sharma's sugar levels. Address the letter to Dr Lisa Smith, Endocrinologist, City Hospital, Newtown.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
**WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

**Notes:**

Mr Brett Collister is a male patient in your general practice.

**PATIENT DETAILS:**

**Name:** Mr Brett Collister (**DOB:** 20 November 1970)

**Height:** 177cm

**Occupation:** Factory foreman

**Social History:** Married, 3 children (18, 16, 13 yrs)

**Hobbies:** Watching football, playing darts, fishing

**Medical History:** No known allergies
Infectious mononucleosis – January 2003

**Treatment Record**

**22/03/14** Productive cough & sore throat for 1 week, green phlegm;
Pt tired, temp (38°C).
**Treatment:** Rest, plenty of fluids, salt water gargles.

**26/05/14** Sore throat – suddenly worse after 3 weeks of intermittent pain & fever; Pt feels 'run-down'.
Tonsils inflamed; temp 38.5°C.
**Treatment:** Prescribed amoxicillin.

**17/08/14** Sore L shoulder – triggered during game of darts 2 weeks previous – ?rotator cuff tear.
Busy at work – feels tired & stressed.
**Treatment:** Prescribed ibuprofen.
R.I.C.E (rest, ice, compression, elevation)
Refer to physio (Mrs Louise Ferguson) for exercise program & treatment for shoulder.

**26/10/14** Sore R knee – pain intermittent, worse going up stairs. No identified trigger.
?osteoarthritis.
Shoulder better.
BP 107/60, HR 78 (reg), Wt 94kg (BMI 30 – overweight).
**Treatment:** Prescribed ibuprofen.
Advised to ↓ weight, ↑ exercise (cycling, swimming).
Refer to physio (as previously) – review in 3 months.



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**4/01/15**

Pt feels tired, 'run-down'; sore eyes, dizzy sometimes (for last 3-4 weeks) – ?orthostatic hypotension.
Overweight, unfit – no adjustment to lifestyle, diet, exercise.
Reports busy at work.
BP 108/61, HR 80 (reg), lungs clear, Wt 93kg (BMI 29.7 – overweight).

**Treatment:** Order blood tests to review cholesterol, blood sugars, etc.

**24/01/15**

Still tired, sore eyes, vision↓.
BP 105/60, HR 78 (reg), lungs clear, Wt 89kg (BMI 28.4 – overweight).
Review of tests organised 4/01/15:

- random glucose 13.5mmol/L (high).
- fasting glucose 7.4mmol/L (high).
- HbA1c 8.5%.
- HDL/LDL ↑ (cholesterol 6.4mmol/L, LDL 4.2mmol/L, HDL 2.1mmol/L, Trig 3.3mmol/L).

**Preliminary diagnosis:** Results indicate DM (diabetes mellitus) Type 2.

**Treatment:** Refer to endocrinologist for assessment and management plan.

## Writing Task:

Using the information in the case notes, write a letter of referral to Dr Grantley Cross, Consultant Endocrinologist, City Hospital, Suite 32, 55 Main Road, Newtown.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
**WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

**Notes:**

Mr Patrick Newton (born on 6 July 1989) is a patient in your General Practice.

**Patient details:**

**Name:** Mr Patrick Newton

**Residence:** 10 Ashwood Street, Stillwater

**Social background:** 25-year-old accountant, single, lives with parents

**21 Feb 2015**

**Subjective**

**Presenting complaint:**

Presentation with 4 month Hx of chronic mild diarrhoea & low-grade intermittent R lower quadrant abdo pain; lethargy, ↓appetite, ↓weight (3kg in 4 months)

**Social/family Hx:**

Smokes 10-15 cigarettes per day
Regular squash player
Uncle has Crohn's disease
↑↑Anxiety and embarrassment relating to symptoms and impact on social participation:
– dietary modification unsuccessful in alleviating symptoms
– recently stopped attending Friday evening squash matches with work colleagues
– has not sought medical advice until now (has attempted to self-manage illness by diet and OTC pain relief)

**Past medical Hx:**

6 month Hx low-grade intermittent joint pain in R & L wrists

**Medications:**

OTC Ibuprofen 200-400mg, 3 or 4 times a day (as required)
No known allergies

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| | |
|---|---|
| **Objective** | T – 36.4°C; P – 80 (regular); Ht – 175cm; Wt – 79kg<br>Abdomen – generalised tenderness, no HSmegaly (enlargement of liver and spleen)<br>Cardiovascular & resp examination – normal<br>Urinalysis – normal<br>FBE ↑ WCC 11.1x$10^9$/L<br>↓ RCC 4.0x$10^{12}$/L<br>↓ Hb 125g/L<br>Faecal occult blood test – positive<br>Mildly elevated CRP (13mg/L) and ESR (14mm/hr) |
| **Assessment:** | ?Inflammatory bowel disease (IBD)<br>?Crohn's disease/ulcerative colitis (UC)<br>No urgent systemic signs |
| **Plan:** | Advise on smoking cessation<br>Counsel on IBD & likely investigations<br>Refer to gastroenterologist for diagnosis & assessment |

## Writing Task:

Using the information given in the case notes, write a letter of referral to gastroenterologist, Dr Jack Thomas, seeking his advice on diagnosis and assessment. Address the letter to: Dr Jack Thomas, Department of Gastroenterology, City Hospital, Main Road, Stillwater.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
**WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

**Notes:**

Your long-term patient, Mrs Welshman, has attended your GP surgery with her daughter. Both are concerned about Mrs Welshman's memory.

**Patient:** Mrs Patricia Welshman (D.O.B.: 28/03/1930)

**Address:** 24 Kenneth St, Newtown

**Marital status:** Widowed, 5 adult children

**Next of kin:** Christine – daughter

**Diagnosis:** Osteoporosis. Dementia (?early stage Alzheimer's)

**Social background:** Widowed 40yrs. Lives alone, children within 10km radius.

**Medication:** OsteVit-D 1000IU, atorvastatin (Lipitor) 20mg mane, ibuprofen (Brufen) 200mg prn, metoprolol (Metrol) 100mg b.d., paracetamol (Panadol) 500mg prn

**Past Medical History:**

**2007–2013** Regular GP visits to this clinic, Pathology, BP – stable

**19 June 2014** Fall – bruised nose only. X-ray – NAD. Will begin to take it easy, slow down.

**27 July 2014** Occupational Therapist (OT) home assessment: Evaluated shower rails, ramp. Bed ok. Rev 4-6mths.
Discussed shower with OT. All ok.
Shower every other day to avoid falls.
Community Support: Home care provided by local council, 1/fortnight.

**14 December 2014** BP 145/85

**Pathology:** FBE, U&Es, LFTs – all NAD

**Lipids:** Total cholesterol 4.8mmol/L (< 5.5)
HDL cholesterol 1.4mmol/L (0.9–2.2)
*LDL cholesterol 2.9mmol/L (< 2.0)
Triglycerides 1.1mmol/L (0.5–2.0)
LDL/HDL 2.1
Chol/HDL 3.4
*Vitamin D < 54 (60-160nmol/L)

**Discussions:** Spare scripts – ?not filling them or taking medication regularly.
Assures me she is taking medication regularly.
Suggested Webster pack (a folder used to store medication on a weekly basis), reluctant, promised to adhere to medication regime.
Rev 2 months, post-pathology.



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**13 February 2015**

**Pathology:** FBE, U&Es, LFTs – all NAD

**Lipids:** Total cholesterol 5.3 mmol/L ( < 5.5)
HDL cholesterol 1.3mmol/L (0.9–2.2)
*LDL cholesterol 3.5mmol/L (< 2.0)
Triglycerides 1.2mmol/L (0.5–2.0)
LDL/HDL 2.7
Chol/HDL 4.1
*Vitamin D < 20 (60-160nmol/L)

**Discussions:** BP 130/80 ✓ encouraged.

Vit D ↓, LDL ↑ – agreed to use Webster pack.

Rev 2 months, post-pathology.

**19 April 2015** BP 130/70, Vit ✓ & Lipids ✓

Medication sorted.

Daughter with Pt, both want to discuss memory issues.

Poor memory noted ++, e.g., forgetting hair dresser, dinner engagements, missing social events. Behavioural changes, decision-making issues. Family concerned.

**Mini memory assessment:**

Poor short-term memory, day & date – several attempts, no result. Month – 3 attempts. Confirmed the year correctly. Quite worried.

Requested further assessment.

Family history of Alzheimer's.

Asked about dementia – explained difference between Alzheimer's (disease – ↑ amyloids in brain) and dementia (symptom). Alzheimer's – common cause of dementia.

More assessments before diagnosis. Referred → Memory Clinic.
Rev, post-assessment.

## Writing Task:

Using the information given in the case notes, write a letter of referral to Dr Jones at the Newton Memory Clinic, 400 Rail Rd, Newtown, to provide him with your brief assessment and request full memory assessment and diagnosis.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
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Read the case notes below and complete the writing task which follows.

**Notes:**

You are a doctor working in the Stillwater Hospital Emergency Department. Today you treated Ms Garcia, who was referred by her General Practitioner (GP), Dr Bradbury.

**Patient Details**

**Name:** Ms Isabel Garcia

**DOB:** 01.01.1995

**Address:** 29 Greenfield Road, Stillwater

**Medical history:**
2007 Fracture R arm
2009 Unexplained weight gain, ?stress
2014 Difficulty sleeping

**Allergies:** Certain washing detergents cause skin irritation.

**Medications:** Doxylamine prn (encouraged not to use).

**Family history:** Mother – breast cancer, age 38.

**Social history:** University student (2nd year).

**Reason for referral:** Suspected meningitis.

**Treatment Record**

**23 May 2015**

**Subjective:**
Painful, stiff joints for 1 wk.
Sensitivity to light.
↑bruising.
Headache, neck stiffness, photophobia, rash.

**On examination:**
Afebrile.
Bruising L arm.
Petechial rash abdomen and legs.
Unable to touch chin to chest when lying supine.

**Tests ordered:**
Full blood count (FBC), renal function, liver function test (LFT), C-reactive protein (CRP), lumbar puncture, blood cultures.



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| **Results:** | White cell count: | $14.0 \times 10^9$/L |
| | C-reactive protein: | 150 |
| | *Lumbar puncture:* | *White cell count 1000 (elevated)* |
| | | Polymorphonuclear (PMN) predominance |
| | Glucose: | 10mg/dl (reduced) |
| | Protein: | 70mg/dl (elevated) |
| | Subsequent microscopy and culture: | Neisseria meningitidis |

**Diagnosis:** Bacterial Meningitis.

**Treatment:** Ceftriaxone 2g IV bd while awaiting lumbar puncture culture results.

Dexamethasone 10mg IV before first dose of antibiotics, then 10mg IV every 6hrs for 4 days.

Following lumbar puncture results: benzylpenicillin 1.8g IV every 4hrs for 5 days.

Pt responding well to treatment.

Department of Human Services notified.

Discussed with family re: ensure family immunised.

**Plan:** Letter to GP, recommend:

Contact close family & friends of Pt:

- Seek medical attention ASAP – observation for any signs of unexplained illness required.
- ?chemoprophylaxis for people in recent close contact with Pt.

## Writing Task:

Using the information given in the case notes, write a letter to Dr Bradbury, the doctor who referred Ms Garcia, to update her on the patient's status and follow-up treatment that may be required in the future. Address the letter to Dr Lorna Bradbury, Stillwater Medical Clinic, 12 Main Street, Stillwater.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
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Read the case notes below and complete the writing task which follows.

**Notes:**

You are a doctor at Newtown Medical Clinic. Mr Barry Jones is a regular patient of yours.

**Patient:** Mr Barry Jones
54 Woods Street
Newtown
D.O.B. 01.04.1972 (age 44)

**Reason for presenting:**
Wants to return to work after back injury – employer supportive

**Medical history:** 1984 – Appendix removed

**Family and social history:**
Married – Susan Jones, 3 children
Work – drives forklift in a large warehouse (requires prolonged sitting / occasional heavy-lifting)

**Current medications:**
Naproxen (non-steroidal anti-inflammatory drug)
Carisoprodol (muscle relaxant, blocks pain)

**Condition history:**

**21/03/15**

**Presentation:** Hurt back lifting heavy box off floor at work.
4 days since initial strain.
No rest, pain worsening.

**X-ray:** No disc problems.

**Diagnosis:** Lower back strain – severe.

**Treatment:** Exercise: walking daily – gradual ↑ time/distance.
Referral to physio.
Prescription: naproxen and carisoprodol.
30 days off work and certificate to give to employer.
To review in 30 days.

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| | | |
|---|---|---|
| **18/04/15** | **Progress:** | Back: Still sore.<br>Moving very stiffly.<br>Physio: Exercises "very painful" but Pt is compliant.<br>Exercise: Walking up to 10 min per day. |
| | **Treatment:** | Extended time off work – 30 days. To review in 30 days. |
| **19/05/15** | **Progress:** | Back: Recovering well – still in pain.<br>Still moving very stiffly.<br>Physio: Attending regular appointments.<br>Exercise: Walking 15-20 mins per day – "very tiring". |
| | **Treatment:** | ↑Naproxen dose.<br>Extended time off work – 30 days. To review in 30 days. |
| **20/06/15** | **Progress:** | Back: Recovering well – still in pain.<br>Moving stiffly but ↑ROM.<br>Pain ↑after 20-30 mins of sitting or lying down.<br>Physio: Still attending appointments. |
| | **Exercise:** | Walking 30 mins per day – "tiring". |
| | **Discussions:** | Pt bored, discouraged, wants to return to work. Restless. |
| | **Treatment:** | Return to work if no lifting & with regular breaks.<br>Letter to OT requesting assessment of workplace (advise on duties Pt can perform, etc.). |

## Writing Task:

Using the information in the case notes, write a letter to Ms Jane Graham, an Occupational Therapist, detailing Mr Jones' situation and requesting an assessment of his workplace. Address the letter to Ms Jane Graham, Newtown Occupational Therapy, 10 Johnston St, Newtown.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
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Read the case notes below and complete the writing task which follows.

**Notes:**

Mrs Katherine Walter is a patient in your general practice.

**History:**

Name: Mrs Katherine Walter
DOB: 26 November, 1975
Height: 170cm
Asthma – since childhood; budesonide (Pulmicort) inhaler, since 28/06/99
Chronic fungal skin infections (both feet) – currently clotrimazole (Canesten)
Moderate family Hx depression (father, sister, aunt, uncle)
Married; two children (8 & 11 yrs)
Home duties
No hobbies or sport
Family (parents, husband's parents & siblings) live in other states

**19/11/14**

**Subjective:** Here for 'check-up'. Seems well, happy, volunteers at her children's school. Reports feeling tired. Asthma controlled, more attacks this year. Fungus on feet flares up periodically – Pt reports no creams seem effective. Overweight.

**Examination:**
BP – 110/95
Heart rate – 76 bpm
Breast check – no palpable mass found
Skin check – no suspicious lesions found
Wt – 82kg
BMI – 28.4

**Tests:**
Pap smear
CBC

**Assessment:** Pt appears well. Needs to ↓weight, ↑exercise. Monitor BMI/fitness/lifestyle.

**Plan:** Advise Pt re lifestyle changes to ↓weight, ↑exercise. Pt to phone for test results in 1 wk. Recommend miconazole (Daktarin) for fungus. R/v appt 3 mths to assess fungal infection, weight and fitness.



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**28/05/15**

**Subjective:** R/v. Pt reports feeling well and energetic. Too busy to come to scheduled r/v 3 mth – didn't think it was necessary. Asthma flared up about two months ago but no attacks since then. Fungus improved. Reports ↑involvement with school (now president of parents' association). Has lost weight, joined gym (trains daily).

**Examination:**
BP – 108/90
Heart rate – 66 bpm
Wt – 69.5kg
BMI – 24
CBC – all results in normal range (results of test 19/11/14)
Pap smear – no abnormalities found (results of test 19/11/14)

**25/07/15**

**Subjective:** Reports feelings of not coping and of wanting to die. Feels tired, but sleeps badly. No energy to complete household tasks, e.g., cooking and cleaning, looking after children.
Feels overwhelmed with responsibilities. Doesn't want to eat.

**Examination:**
BP – 120/90
Heart rate – 78 bpm
Wt – 50kg
BMI – 17.3
Temp – 37.5°C

**Assessment:** Depression – severe / ?bipolar disorder. Requires urgent treatment.

**Plan:** Refer to psychiatrist for urgent assessment and treatment for depression/ bipolar disorder and suicidal thoughts. Contact husband to discuss child care, household maintenance, etc.

## Writing Task:

Using the information in the case notes, write a letter of referral to the psychiatrist, Dr M Jones, 23 Sandy Road, South Seatown.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
## WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

**Notes:**

Mrs Mary Clarke (born on 17 September 1960) is a patient in your General Practice.

**Patient details**

**Name:** Mrs Mary Clarke

**Address:** 26 Marine Drive
Riverside

**Social background:**
54-year-old office clerk
Married, lives at home with husband and 20-year-old son
Smokes 30-35 cigarettes per day (>30 yrs)

**Family/medical history:**
Mother died 66 y.o. – laryngeal carcinoma
Father (coal miner) died 54 y.o. – mining-related lung disease
Nil medication
No known allergies

**04.07.15** Patient presented with sore throat, body aches, fever and cough.

**Prescription:** Augmentin (penicillin)

**22.08.15**

**Presenting complaint:**
7-week Hx of dry non-productive cough (no haemoptysis)
Cough commenced with flu-like symptoms → cleared with Augmentin
Associated mild shortness of breath (esp. at night) and "strange sensation of heaviness" in chest
Nil fever, night sweats or rigors
Exercise tolerance OK – chores, shopping, could walk up 2 sets of stairs

**Examination:** T: 36.7°C, P: 80 regular, Ht: 165cm, Wt: 68kg
Respiratory exam – signs of consolidation associated with monophonic wheeze in R mid-zone
No cyanosis/dyspnoea/ascites
No hoarse voice/Horner's syndrome
No cervical lymphadenopathy
No hepatosplenomegaly/bone pain
Systems review – GIT & CV normal
Sputum cytology – normal
Chest X-ray and CT – R middle lobe atelectasis, enlarged R hilum





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**Assessment:** ?Bronchogenic carcinoma

**Plan:** Counselled on potential diagnosis and need for further investigations

Refer to thoracic surgeon for follow-up investigations (bronchoscopy, biopsy) and assessment

## Writing Task:

Using the information given in the case notes, write a letter of referral to the thoracic surgeon, Dr Penny Clifton, seeking follow-up investigations and assessment. Address the letter to: Dr Penny Clifton, Department of Cardiothoracic Surgery, Central Hospital, Main Street, Stillwater.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
**WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

**Notes:**

Mrs Lucy Clarke is a patient in your General Practice.

**Patient details**

**Name:** Mrs Lucy Clarke

**DOB:** 11 March 1951

**Residence:** 23 Mountain Drive
Coast City

**Social background:** 64-year-old retired office clerk
Independent, lives at home with husband
Non-smoker, social drinker

**20.09.15**

**Presenting complaint:**

1 week history central crushing chest pain on exertion (3x, <15 mins duration)
Associated dyspnoea, radiation of pain down L arm
Relieved by rest
No palpitations, no orthopnoea (difficulty breathing on lying down), no paroxysmal nocturnal dyspnoea (difficulty breathing at night)
Pt anxious – believes had a "heart attack"

**Past medical history:**

2001 – diabetes mellitus (DM) Type II (currently stable)
2003 – hyperlipidaemia
2005 – hypertension (HT)

**Medications:** Sitagliptin (Januvia) 100mg per oral (p.o.) mane
Insulin (NovoMix30) 25 units subcutaneously (s.c.) b.d.
Atorvastatin (Lipitor) 40mg p.o. mane
Irbesartan (Avapro) 75mg p.o. mane

**Family history:** Mother – acute myocardial infarction (MI) at 57 y.o.; died of ischaemic stroke at 59 y.o.



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**Examination:** T – 36.7°C, P – 80 regular, Ht – 164cm, Wt – 65kg
No peripheral oedema
Systems review – normal
Resting ECG – normal

**Provisional diagnosis:**
Unstable angina

**Plan:** Hospital admission for urgent assessment
Referral to Emergency Department cardiologist for update & further management
Counsel patient – advised on serious risk of MI

## Writing Task:

Using the information in the case notes, write a letter of referral to the Emergency Department cardiologist, Dr Smith. Address the letter to: Dr David Smith, Cardiologist, Emergency Department, Main Hospital, Coast City.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

**TIME ALLOWED:** **READING TIME: 5 MINUTES**
**WRITING TIME: 40 MINUTES**

Read the case notes below and complete the writing task which follows.

**Notes:**

Your patient, an 81-year-old woman, recently had a right total knee replacement (R.TKR) on 25/02/2015. She is being discharged today.

**Patient:** Ms Betty Johnson

**Address:** 12 Merry Street, Stillwater

**Marital Status:** Widowed

**Past Medical History:** Aortic valve replacement & pacemaker 2010
Osteoarthritis since 2011 — pain & immobility ↑ past 3yrs
For R.TKR Feb 2015: full blood work, typing & cross matching, X-rays, ECG etc.

**Regular Medication (25/2/15):**
Paracetamol 665mg 2 tabs tds
Warfarin 3mg mane – ceased 5 days preoperatively, started Clexane (enoxaparin sodium – anticoagulant)

**Social Background:** Widowed 1986. Lives alone. 4 children

**Post Op:**

**25/02/15 11:30am** Returned to ward following R. TKR.
Vital signs — BP 115/70, P 82, R 16, T 36.9°C.
Circulation observation good, knee ↑ on pillow.
Hb 80g/l → IVT Transfusion.
IV cephalothin 1g qid for 24 hours.
Increase regular oral paracetamol (1g qds).
Patient Controlled Analgesia (PCA) — morphine ✓ effective.
Wound — nil ooze.

**26/02/15** Wound — good, sponged.
Restart warfarin 5mg today.
s/c Clexane 80mg given for anticoagulation.
Cease PCA. Start oxycodone 5 – 10mg prn.
Pathology: FBE, U&Es, Liver Function Tests (LFTs), Hb.
Path results ✓, Hb 100g/l → commence Feratab (iron sulphate) 300mg mane.

**27/02/15** s/c Clexane 80mg.
Start warfarin 5mg nocte.
Removal of (R/O) dressing, wound good, R/O alt. clips on 03/03/15.

**28/02/15** Crutches, short walks. Wound good, afebrile.
s/c Clexane 80mg given.



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| | |
|---|---|
| **01/03/15** | s/c Clexane 80mg given. |
| **02/03/15** | X-rays, bloods ✓, INR — 3.0, Hb 111g/l, ECG – no abnormalities.<br>Managing w/ min assistance.<br>Cease Clexane. |
| **03/03/15–05/03/15** | Wound clean, R/O alt clips tomorrow. Mobility good. Obs ✓. |
| **06/03/15** | R/O remaining clips. Pathology ✓. Transfer to rehab today. |
| **Rehab:** | |
| **07/03/15** | Admission complete – stable. Circ ✓. Mobility, crutches good. |
| **08/03/15–13/03/15** | Mobility, frame use, trial stick, pool, gentle exercises = good. Showering w/ min assistance.<br>Path & X-ray. |
| **14/03/15** | Path ✓, INR — 3.8.<br>↓ warfarin 4mg nocte, Hb — 112g/l, ↓ Feratab 150mg mane. |
| **15/03/15–19/03/15** | Uneventful – gradually ↑ independence.<br>Wound good. Obs ✓. Physio exercises good. Home list provided. |
| **21/03/15** | No cardiac issues.<br>Discharged w/ home nursing assistance (personal hygiene, home care). Wound exposed, shower w/ min assist. Stick / frame prn.<br>Discharge medication: warfarin 4mg nocte, Feratab 150mg mane, paracetamol 1g qds, oxycodone 5–10mg prn.<br>Rehab appt in 2 weeks.<br>Advised to see local doctor in 1 week, referral for local doctor — suggest repeat FBE, INR. |

## Writing Task:

Using the information given in the case notes, write a letter of referral to Ms Johnson's local doctor, Dr Tony Jones, to update him on her condition following her recent surgery and discharge from rehab. Address the letter to Dr Tony Jones, Private Practice, 12 New Street, Stillwater.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**



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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
## WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

**Notes:**

Mr James Seymour is a 60-year-old man presenting in your general practice with a swollen left large toe.

**Patient details:**

| | |
|---|---|
| **Name:** | James Seymour |
| **Residence:** | 4 Powlet Drive, Clayfield |
| **DOB:** | 19/09/53 (Age 60) |
| **Social history:** | Retired academic (computer science)<br>Divorced, no children, lives alone<br>Non-smoker since 1994<br>Heavy drinker 5-6 beers and 3 wines/day |
| **Observations:** | BP 115/70mmHg, HR 68, RR 18, T 37.4°C |
| **Allergies:** | Nil known |
| **FHx:** | Father – rheumatoid arthritis (RA) – 28 yrs old. Died 75yrs.<br>Mother – smoker, died chest infection aged 71yrs.<br>Grandparents' history unknown, died when old. |
| **PMHx:** | Appendicectomy 1963<br>Childhood – recurrent bronchitis<br>Annual influenza vaccine<br>Regular episodes of inflammation (?gout 1st toe) since 2010 – consulted several doctors |
| **Medication:** | Colchicine (Lenigout) – 500mcg 2 tabs (stat on attack) then 1 tab each 2/24 until relief. Total dose ≤ 6mg in 4 days.<br>Indomethacin (Indocid) – 25mg 2 tabs, twice/day.<br>On allopurinol after last acute attack – after several mths w/o symptoms ceased meds (a couple of mths before current episode). |

**Treatment record:**

**25/04/14**

~ 4 wks into current bout of gout.
Colchicine started 2 wks into bout, only taken at sub-therapeutic levels.
Indocid taken erratically.
3rd bout in 8 mths.
No allopurinol for a couple of mths.
Modifies diet to ↓purines. Sometimes wakes at night.
Given father's Hx Pt wants referral to rheumatologist to exclude RA.
Pt thinks gout meds not working (unlikely).

**On examination:** Moderately inflamed, red first L toe. V painful – Pt irritated. No evidence of involvement of other joints.

Pt V insistent on possibility of RA; poor compliance with gout management much more likely.

**Treatment:**

- Encouraged to comply with gout meds:
  - resume full dose colchicine.
  - resume full dose indomethacin. Cease either if gastrointestinal (GI) side effects (diarrhoea from colchicine; upper GI upset from indomethacin).
- Regular paracetamol (4g/day for 3 days, then prn).
- Take oxycodone 5mg bedtime only if sore and can't sleep; try to cease ASAP.
- Improve dietary compliance and ↓ alcohol intake.
- X-ray L foot, FBE, ESR, LFT, U&E, SUA, CRP.
- Rev. 1/52 to discuss results & referral.

**03/05/14**

X-ray – minor degenerative changes of L first metatarsophalangeal joint.
FBE: *MCH 32.3pg (Ref Range: 27.0 – 32.0). All other NAD.
*Urate 0.48mmol/L (Ref Range: 0.18 – 0.47mmol/L).
*CRP 6.0mg/L (Ref Range: < 3.0).
Gout episode subsiding.
No drug side effects apart from brief diarrhoea.
Only needed night time oxycodone 3 nights.

**Provisional Diagnosis:**

Gout.

**Treatment:**

- Discussed ?synovial fluid sample stat next episode.
- Start allopurinol now, long term; reinforce messages re: diet & alcohol.
- Referral to Rheumatologist on patient's insistence with copy of pathology results.

## Writing Task:

Using the information given in the case notes, write a letter to Dr Malcolm Still, Rheumatologist at 5 Grant St, Fairmont, for further treatment or investigations.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

# Medicine Letter 9

Read the case notes below and complete the task that follows.

**WRITING SUB-TEST**
DOCTORS

**Time allowed: 5 minutes reading time (no writing), 40 minutes writing time**

Mrs. Larissa Zaneeta is a patient in your general practice. Read the case notes below and complete the writing task that follows.

**CASE NOTES**

**Mrs. Larissa Zaneeta**
38 year old marketing manager, married, one child (four-year-old boy).

Past history unremarkable. No medications

**11/07/05**

Complains of tiredness, difficulty sleeping for 2 months due to work stress
Plans another child in 12 months, currently on oral contraceptive pill (OCP)

**O/E:** Appears pale, tired and slightly restless
BP 140/80
No abnormal findings

**Assessment**: Stress-related anxiety

**Plan:** advised relaxation techniques, reduce working hours, prescribe sleeping tablets tds

**15/08/06**

Stopped OCP 4 months earlier, still menstruating
Worried
Sleep still difficult, work stress unchanged, not possible to reduce hours

**O/E:** Tired-looking, slightly teary

**Assessment:** Work stress, growing anxiety failure to conceive

**Plan:** discussed nature of conception – takes time, patience
discussed frequency sexual intercourse
discussed methods – temperature / cycle

**18/01/07**

expressed anxiety re failure to conceive, says she's "too old"
sleep still a problem

**O/E:** crying, pale, fidgety
Vital signs / general exam NAD
Pelvic exam, pap smear

**Assessment:** as per previous consultation

**Plan:** 1-2 Valium b.d.
Suggested she re-present next week accompanied by wife.

**25/01/07**

Mr. Zaneeta very supportive of having another child
No erectile dysfunction, libido normal
Mrs. Zaneeta unchanged

**O/E:** Mr. Zaneeta normal

**Plan:** Check Mr. Zaneeta's sperm count

**02/02/07**

Sperm count normal

**Plan:** Refer for specialist advice

**WRITING TASK**

**Using the information in the case notes, write a letter of referral to Dr Elvira Sterinberg, a gynaecologist at 123 Church St Richmond 3121. The main part of the letter should be approximately 180-200 words long.**

**DO NOT use note form in the letter; expand the case notes where relevant into full sentences.**

# Medicine Letter 8

## OET Practice Writing Test

Read the case notes below and complete the task that follows.

**WRITING SUB-TEST**
DOCTORS

**Time allowed: 5 minutes reading time (no writing), 40 minutes writing time**

Ms Ann Howard is a patient in your general practice. Read the case notes below and complete the writing task that follows.

**CASE NOTES**

**Mrs. Ann HOWARD**
36 years old, married 3 children

**Past history**

Ovarian cystectomy and appendicectomy
Early October 2006 last menstrual period
18/12/06 – left lower abdominal pain
09/01/07 – vaginal bleeding, abdominal cramps. Presented hospital emergency dept
? spontaneous abortion

20/01/07

**Subjective**

Reported yesterday sudden onset L lower abdo pain, relieved by Valium
Today pain persists, sharp and constant, worse sitting up, walking or bending
No vomiting or nausea, no urinary or bowel symptoms, no weight loss, no change of bowel habits

**Objective:**

Not distressed
Pulse 96 reg, BP 140/80
Very tender on light palpation L lower quadrant abdomen
Vague mass palpable

Arranged tests: pregnancy test, Full Blood Exam, ESR

21/01/07

Pain persists but less
No bowel motion for 3 days when passed hard stool coated with bright red blood

22/01/07

**Subjective**:

Pain worse after eating

**Objective:**

Moderately distressed, abdomen tense
Haemoglobin 9.3 g/dl. Mild left shift
Quiet bowel sounds
No bowel action or flatus

**Assessment**: Early bowel obstruction ? diverticulitis ? carcinoma

**Plan**: Referral Dr. Jose Jiminez surgeon

**WRITING TASK**

**Using the information in the case notes, write a letter of referral to Dr Jiminez, a surgeon at Melbourne Private Hospital 19 Grange Road Melbourne 3000. The main part of the letter should be approximately 180-200 words long.**

**DO NOT use note form in the letter; expand the case notes where relevant into full sentences.**

# Medicine Letter 7

## OET Practice Writing Test

Read the case notes below and complete the task that follows.

**WRITING SUB-TEST**
DOCTORS

**Time allowed: 5 minutes reading time (no writing), 40 minutes writing time**

Ms Janet Bird is a patient in your general practice. Read the case notes below and complete the writing task that follows.

**CASE NOTES**

**Ms Janet BIRD**
16 yo girl

**Past history**

Unremarkable, no medications

**Social History**

Attends local secondary school, Year 11, lives parents, younger brother

11/11/07

**Subjective**

Presented alone
Constipation 3 months, 1 X firm bowel action every 4-5 days
Diet includes 2 tablespoons bran in morning, has tried laxatives
Otherwise well

**Objective:**

Ht. 172 cms Wt. 52 kgs.
Pulse 73 reg, BP 100/50
Abdomen lax, no masses

Pt. Requested prescription for "strongest" laxative. Request refused. Advice re ↑ vegetables, fibre and fluids.

28/12/07

**Subjective**:

Presents with mother. Mother concerned re Janet's lack of appetite and weight loss. Family arguments about the situation.

**Objective:**

Pale, thin. Wt. 47 kgs.
BP 100/60 lying and standing
Abdomen and urinalysis both unremarkable

**Plan**: Review Janet alone, Tests Full Blood Exam, Thyroid Function, Liver Function

05/01/08

**Subjective**:

Janet complains parents are "over-reacting". Feels her ideal weight is 40 kgs.
Denies vomiting

Test results: normal

**Assessment**: Anorexia nervosa

**Plan**: Referral Dr. Suzanne O'Brien, psychiatrist

**WRITING TASK**

**Using the information in the case notes, write a letter of referral to Dr O'Brien, a psychiatrist at 67 Sigmund Street Brighton 3186. The main part of the letter should be approximately 180-200 words long.**

**DO NOT use note form in the letter; expand the case notes where relevant into full sentences.**

# Medicine Letter 6

## OET Practice Writing Test

Read the case notes below and complete the task that follows.

**WRITING SUB-TEST**
DOCTORS

**Time allowed: 5 minutes reading time (no writing), 40 minutes writing time**

Mr Zu is a patient in your general practice. Read the case notes below and complete the writing task that follows.

**CASE NOTES**

03/01/07

**Mr Jing ZU**
72 yo man.

**Past history**

Hypertension 18 years
Ischaemic heart disease 10 yrs
Acute Myocardial Infarction 1999
Congestive Cardiac Failure (CCF) 5 yrs

**Family history** unremarkable

**Medications**

Lasix 40mg mane, Enalapril 10mg mane, Slow K TT bd, Nifedipine 10mg tds, Anginine T sl prn

**Social History**

**Job:** retired school teacher
**Home:** married
**Activities:** gardening
smoking: no

**Subjective**

Angina on exertion – gardening, relief with rest and Anginine
Sleeps two pillows, no orthopnoea
Mild postural dizziness

**Objective:**

Thin, looks well.
Pulse 84 reg, BP 160/90 lying, 145/80 standing
Jugular Venous Pressure (JVP) + 3 cm
Apex beat not displaced
S1 and S2 no extra sounds nor murmurs
Chest - Bilateral basal crepitations
Abdomen – normal
Ankles mild oedema, pulses present

**Assessment:** Stable CCF, angina

**Plan:** Watchful monitoring

15/01/07

**Subjective**:

↑ dyspnoea, orthopnoea (sleeps on 4 pillows)
↑ ankle oedema
no chest pain

**Objective:**

BP 140/90
JVP + 6 cm
Chest crepitations to mid zones
Heart S1 and S2
Ankles oedema to knees

**Assessment**: Deteriorating CCF ? cause

**Plan**: ECG, ↑ Lasix 80 mg mane, R/V 2 days

19/01/07

**Subjective**:

Dyspnoea "feels a bit better"
Angina 10 min episode on mild exertion yesterday

**Objective:**

JVP + 4 cm
Chest fewer crepitations to mid zones
ECG - ? ischaemic changes anterolaterally

**Assessment**: ischaemic heart disease

# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
## WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

**Notes:**

You are a general practitioner. Ms Sarah Day, 20 years old, has been attending your practice since her early childhood.

**Patient:** Ms Sarah Day, D.O.B. 29.07.1997

**Past medical history:**
**Jul 2001:** varicella
**Apr 2002:** measles
**Jan 2004:** fractured ulna
**Jun 2006:** URTI
**Sep 2008:** plantar warts
**Dec 2011:** dysmenorrhoea
**Apr 2013:** teenage acne
**Aug 2014:** pre-menstrual syndrome
**Jun 2015:** exam-related anxiety
**Nov 2016:** oro-facial herpes simplex virus

**Social background:**
University student – not happy with course of study
Part-time job – McDonald's
Irregular hours, 5-8 hrs sleep/night
Smokes, drinks moderately
Lives at home with parents, boyfriend for 7 months

**Medications:** Oral contraceptive pill (prescribed April 2013) cyproterone acetate/ethinylestradiol – mane

**06.12.17** Pt presented with mother complaining of unilateral headache – occipital, temporal extending to vertex, dizziness/loss of balance, with nausea and anxiety. Visual disturbances. Better when lying down. Symptoms not affected by red wine, alcohol, chocolate, cheese, sunlight. Not related to menstrual cycle or stress.

Pt reports workload stress, pressure of assignments & exams.

**Episodes:** x2 in past 2 weeks.

**Pain:** Steady, not throbbing.

**Onset:**
Rapid
Aura
Symptoms last 1-2 days (severe for several hours).
No family history of headaches/migraines.

**Examination:** No abnormal neurological signs.

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| | |
|---|---|
| **Treatment:** | Education: rest/relaxation/meditation, adequate sleep, regular eating.<br>During attack: sleep, dark room, ice.<br>Metoclopramide 10 mg – orally → 20 mins later paracetamol 1 g – 4 hrly (max 4 g/24 hrs). |
| **04.01.18** | Attacks more frequent. Prescribed medications ineffective.<br>Pain felt in neck.<br>Anxiety becoming worse.<br>Dizziness during attacks.<br>Vomited on one occasion.<br>Mother reports Pt becoming afraid to leave house in case attack occurs.<br>No identifiable triggers.<br>Dark room, ice – no effect. |
| **Treatment:** | Eletriptan 40 mg – orally on attack.<br>Ibuprofen 400 mg – 6 hrly (max t.d.s.). |
| **31.01.18** | Pt complained of drowsiness and diarrhoea since commenced on eletriptan.<br>No improvement in symptoms. Anxiety worse – Pt describes as 'panic' accompanying symptoms.<br>Mother concerned daughter becoming depressed: withdrawn, housebound, losing interest in usual activities & boyfriend, oversleeping, comfort eating → weight gain.<br>B.P 120/80 |
| **Treatment:** | Amitriptyline 25 mg – b.d. |
| **24.02.18** | Pt presented alone. Complains of numbness and tingling (paraesthesia) in fingers 4 & 5 left hand.<br>No improvement in symptoms.<br>Pt reported car accident Jun 2017. Not previously disclosed as wishes to keep from mother.<br>Sustained 'whiplash injury'.<br>No treatment sought at time.<br>Referral to neurologist, for investigation and management. |

## Writing Task:

Using the information in the case notes, write a letter of referral to Dr Robert Edwards, a neurologist. Outline Ms Day's relevant history and request further investigations and management. Address the letter to Dr Robert Edwards, Rushford Hospital, 765 Long Gully Road, Littletown.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do <u>not</u> use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

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# OCCUPATIONAL ENGLISH TEST

## WRITING SUB-TEST: MEDICINE

## TIME ALLOWED: READING TIME: 5 MINUTES
## WRITING TIME: 40 MINUTES

Read the case notes below and complete the writing task which follows.

**Notes:**

You are a doctor at Stillwater Private Practice. You are examining a 70-year-old woman who believes she has worsening arthritis.

**Patient details**

| | |
|---|---|
| **Name:** | Mrs Carol Potter |
| **DOB:** | 30.12.1947 |
| **Address:** | 21 Gumtree Road<br>Stillwater |
| **Medical history:** | 2008 Osteoarthritis (OA) – mostly of hands & knees<br>2015 Hypertension (HT) – well controlled<br>2016 Skin cancer removed<br>2016 Insomnia – 2 years, intermittent<br>Urinary tract infections (UTIs) – intermittent |
| **Medications:** | Ramipril 5mg daily<br>Panadol Osteo (extended release paracetamol) 2 tablets t.d.s.<br>Temazepam 10mg nocte p.r.n. |
| **Family history:** | Mother – breast cancer |
| **Social background:** | Administrative assistant (retired) |
| **Presenting complaint:** | ↑ Pain in L knee with walking for last 12 months. Now quite severe – not relieved by regular Panadol Osteo. Pain can even occur at rest after a long walk. |

**Treatment record**

**23.02.18**

| | |
|---|---|
| **Subjective:** | No joint swelling/redness<br>No recent injury to knee<br>R knee – some pain on walking, not nearly as bad as L knee |
| **On examination:** | Evidence of ↓ ROM of L knee due to pain<br>No swelling<br>Tender to pressure along joint |
| **Treatment:** | Referral for X-ray of L knee, blood tests<br>Review appointment to discuss results tomorrow<br>Prescribe pain relief – naproxen 250mg b.d. |

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**24.02.18**

**Test results:**

- X-ray: Evidence of severe OA in L knee – osteophytes and loss of joint space
  Patella appears normal
  No evidence of fractures
- Blood: FBE, UEC (normal)

**Assessment:** Likely worsening OA

**Treatment:** Arrange physiotherapy
↑Analgesia
Referral for surgical consultation – ? knee joint replacement

## Writing Task:

Using the information given in the case notes, write a letter of referral to Dr Waters, a surgeon at Stillwater Private Hospital, for a surgical consultation. Address the letter to Dr Leigh Waters, Surgeon, Stillwater Private Hospital, 54 Main Street, Stillwater.

**In your answer:**

- **Expand the relevant notes into complete sentences**
- **Do not use note form**
- **Use letter format**

**The body of the letter should be approximately 180–200 words.**

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# Medicine Letter 10

**Patient:** Anne Hall (Ms)

**DOB:** 19.9.1965

Height: 163cm Weight: 75kg BMI: 28.2 (18/6/10)

**Social History:** Teacher (Secondary – History, English)

Divorced, 2 children at home (born 1994, 1996)

Non-smoker (since children born)

Social drinker – mainly spirits

**Substance Intake:** Nil

**Allergies:** Codeine; dust mites; sulphur dioxide

**FHx:** Mother – hypertension; asthmatic; Father – peptic ulcer

Maternal grandmother – died heart attack, aged 80

Maternal grandfather – died asthma attack

Paternal grandmother – unknown

Paternal grandfather – died 'old age' 94

**PMHx**: Childhood asthma; chickenpox; measles

1975 tonsillectomy

1982 hepatitis A (whole family infected)

1984 sebaceous cyst removed

1987 whiplash injury

1998 depression (separation from husband); SSRI – fluoxetine 11/12

2000 overweight – sought weight reduction

2002 URTI

2004 dyspepsia

2006 dermatitis; Rx oral & topical corticosteroids

**18/6/10** PC: dysphagia (solids), onset 2/52 ago post viral(?) URTI

URTI self-medicated with OTC Chinese herbal product – contents unknown

No relapse/remittent course

No sensation of lump

No obvious anxiety

Concomitant epigastric pain radiating to back, level T12

Weight loss: 1-2kg

Recent increase in coffee consumption

Takes aspirin occasionally (2-3 times/month); no other NSAIDs

Provisional diagnosis: gastro-oesophageal reflux +/- stricture

**Plan:** Refer gastroenterologist for opinion and endoscopy if required

## Writing task

Using the information in the case notes, write a letter of referral for further investigation and definitive diagnosis to the gastroenterologist, Dr Jason Roberts, at Newtown Hospital, 111 High Street, Newtown.